

A HEALTH CURRICULUM · STUDY DECK

VOL. 01 · 2026

BODY & MIND

OS

A health operating system for Indian families — *60 topics, 8 phases, one
playbook.*

SELF-PACED · REFERENCE-FRIENDLY

ICMR · WHO · NIH · NFHS-5 · LANCET

Eight phases. Sixty topics.

A read-once, return-often reference for the whole family.

01

Foundations of Health

06 TOPICS · 01-06

How disease really builds — and the metrics that matter.

02

Nutrition Fundamentals

12 TOPICS · 07-18

Calories, protein, carbs, fats — in the Indian thali.

03

Gut & Metabolic Health

07 TOPICS · 19-25

Microbiome, fibre, insulin resistance, diabetes.

04

Movement & Fitness

07 TOPICS · 26-32

Cardio, strength, walking, recovery.

05

Sleep, Stress & Mind

06 TOPICS · 33-38

Sleep architecture, breathwork, habits.

06

Indian-Specific Challenges

07 TOPICS · 39-45

Vitamin D, PCOS, premature CVD, women, kids, elders.

07

Screening & Toolkit

07 TOPICS · 46-52

What to test, when, and how to read the report.

08

Family Health OS

08 TOPICS · 53-60

Captain, vault, calendar, insurance, capstone.

PHASE 01 / 08

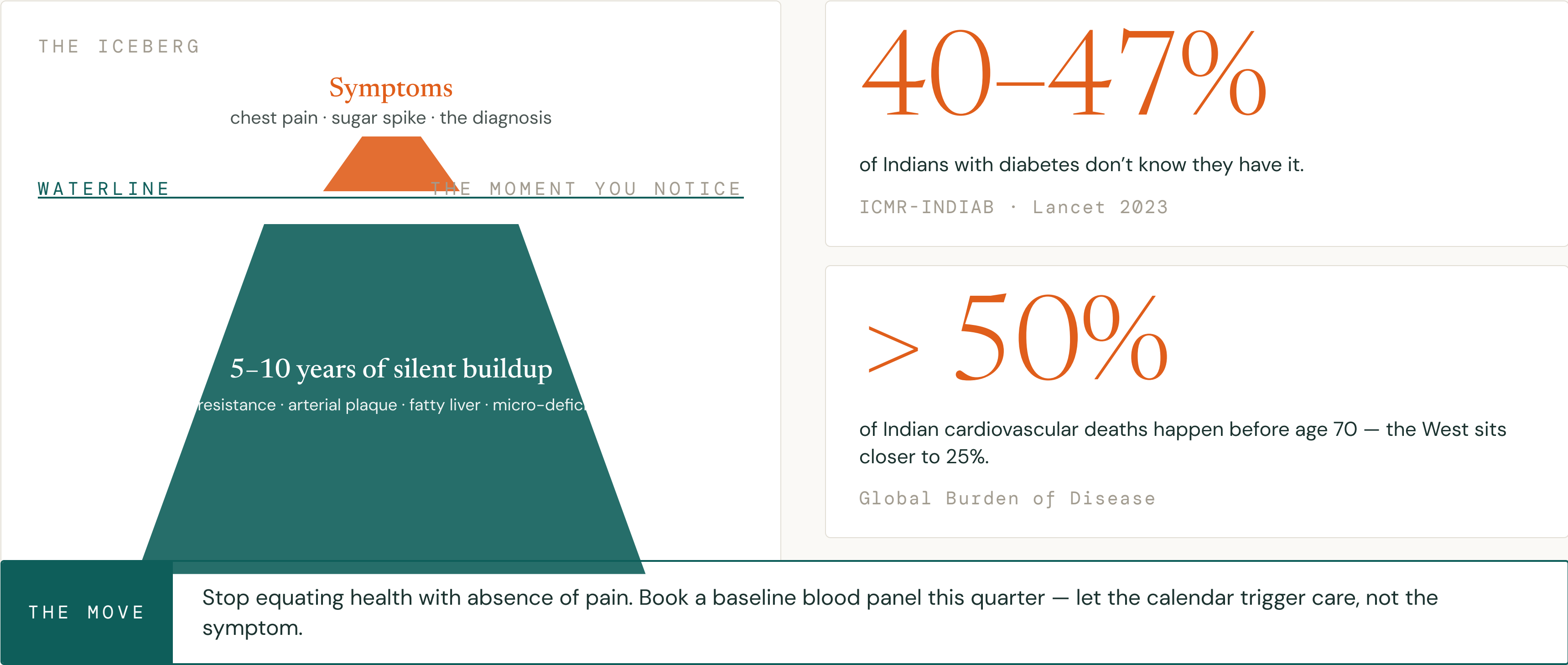
BODY & MIND 05

Foundations of Health

TOPICS 01 — 06 *How disease actually builds, and what to measure before anything hurts.*

What “preventive health” actually means

Most serious disease builds silently for 5–10 years before the first symptom shows up.



The six pillars of health

Health rests on six pillars. Not one. Not two. Six. And you can’t out-exercise a missing one.

01

Nutrition

Fuel and building blocks.

SEE → Phase 2

02

Movement

Cardio, strength, daily steps.

SEE → Phase 4

03

Sleep

7–9 hours, non-negotiable.

SEE → Phase 5

04

Stress & Mind

Cortisol is metabolic.

SEE → Phase 5

05

Social

Loneliness ~ 15 cigarettes/day.

SEE → Phase 8

06

Environment

Air, light, ergonomics.

SEE → Phase 4&7

THE TRAP

You can’t out-exercise a bad diet. You can’t out-supplement bad sleep. You can’t meditate away loneliness.

THE MOVE

Score yourself 0–10 on each pillar tonight. The lowest score is your next 30 days of work — not the highest.

How the Indian body is different

The “thin-fat Indian” – visceral fat at a normal BMI – needs tighter cut-offs than the Western chart.

CUT-OFFS COMPARED

MARKER	WESTERN	INDIAN
BMI · overweight	25	23
Waist · men	102 cm	90 cm
Waist · women	88 cm	80 cm
Onset of CVD	baseline	~10 yrs earlier

ICMR consensus on Asian Indian cut-offs

THE PARADOX

An Indian man at BMI 24 can already carry fatty liver, dyslipidemia and pre-diabetes – while a Western chart calls him “normal.”

WHY VISCERAL FAT MATTERS

Fat around the organs — not under the skin — drives insulin resistance, inflammation, and early heart disease. It hides on the scale and shows up on the waistband.

THE MOVE

Measure waist once a month — fasted, at the navel. Men > 90 cm or women > 80 cm is a louder warning than any reading on the scale.

The Indian disease burden, mapped

Six numbers every Indian adult should know. And one that ties them all together.

101 M

diabetics

ICMR-INDIAB / Lancet 2023

136 M

pre-diabetic

ICMR-INDIAB / Lancet 2023

11%

CVD prevalence

Pooled adult data

7–20%

women with PCOS

JAMA Network Open

46.5%

Vitamin D deficient

Metropolis Labs · 22 L tests

57%

women anaemic (15–49)

NFHS-5

THE SUM

~65% of all deaths in India are from non-communicable diseases — most of them lifestyle-driven.

THE MOVE

Circle the two numbers above that match your family’s risk profile. Read every remaining topic with those two in mind.

Sickcare vs healthcare

Late diagnosis isn't a clinical failure. It's a scheduling failure.

SICKCARE · REACTIVE

Triggered by symptoms.

Disease often years in by the time you visit.

Specialist-led · fragmented · no single view.

Reports scattered across 5 clinics.

Treatment late, expensive, often irreversible.

HEALTHCARE · PROACTIVE

Triggered by the calendar.

Annual baselines, age-appropriate screening.

One family health captain owns the trend.

Records live in one vault — the OS.

Catches drift early — while reversible.

THE MOVE

Name one family health captain this week. Their first job: a shared calendar with annual baselines plus a 30-minute Sunday review window.

Health metrics that actually matter

Two buckets to track. One bucket to ignore entirely.

BUCKET 1 · HOME

Track monthly

Resting heart rate

Blood pressure (averaged)

Waist circumference

Grip strength

VO₂ max (12-min run or watch)

BUCKET 2 · ANNUAL PANEL

Test on schedule

HbA1c · fasting glucose

Lipid panel + ApoB

Vitamin D, B12, ferritin

TSH, hs-CRP

LFT · KFT

IGNORE

Vanity metrics

~~Weight alone~~

~~BMI without waist~~

~~Single BP reading~~

~~App “wellness scores”~~

~~Watch calorie burn~~

THE MOVE

Build a one-page dashboard with date columns. A single number means nothing — the direction it moves over 2–3 years is the warning signal.

PHASE 02 / 08

BODY & MIND 05

Nutrition Fundamentals

TOPICS 07 — 18 — *Twelve topics on what to put on the Indian plate, and what to keep off it.*

Calories and energy

Physics is real. Biology is also real. Both extremes are wrong.

PHYSICS

Energy in vs energy out still holds.

Consume more than you burn over time, you gain weight. Thermodynamics is not debatable.

BIOLOGY

But hormones decide what you do with those calories.

Insulin stores fat. Leptin signals fullness. Ghrelin drives hunger. Sleep, processing and fibre change all three.

300 KCAL FROM DAL VS 300 KCAL FROM PARLE-G

Identical on the scale. Wildly different effects on your hunger, your insulin, and what you eat for the rest of the day.

THE MOVE

Don't count daily calories. Build a default plate (Topic 12) and trust the scale on a four-week trend — not daily, not weekly.

Protein – India’s quiet deficiency

60–75% of Indian protein comes from cereals alone – incomplete amino acids, low satiety, low repair.

DAILY TARGET

1.2–1.6

grams per kg body weight

FOR A 70 KG ADULT

84–112 g/day

VEG SOURCES

- Dal, rajma, chana
- Paneer, curd
- Soya chunks, tofu
- Sprouts, peanuts
- Whey if needed

NON-VEG SOURCES

- Eggs (whole)
- Chicken breast
- Fish, prawns
- Small fish — bonus calcium
- Mutton (lean)

WHY IT MATTERS

Highest satiety per calorie of any macro. Highest thermic effect — ~25% burned digesting. Raw material for muscle, enzymes, antibodies and hormones.

THE MOVE

A fist-sized protein source at every meal — breakfast included. Track for one week and you’ll be surprised how low the number actually is.

Carbohydrates – not the enemy

Isolated low-fibre carbs are. The Indian thali was engineered right.

THE CARB LADDER

Best	millet s · ragi, bajra, jowar
Good	parboiled rice, whole-grain roti, oats
Okay	white rice + dal + sabzi
Limit	white bread, sugary chai, fruit juice

WHAT MATTERS

Glycemic load – not glycemic index.

A full thali (rice + dal + sabzi + dahi) spikes glucose far less than rice alone. Protein, fat and fibre buffer the spike.

ONE RULE

Never eat a carb alone.

THE MOVE

Eat vegetables or protein first at every meal. Swap one rice meal a day for millets or parboiled rice.

Fats — the kitchen ranking

Fat doesn't make you fat. Industrial trans fats and burnt oil do.

GOOD · ROTATE

Mono-unsat & PUFA

Mustard, groundnut, sesame, olive (cold).
Small fish, flax, chia, walnuts for omega-3.

FINE · MODERATE

Saturated

Ghee, coconut, dairy fats. Keep total visible
oil & ghee within ~25–30 g/day.

LIMIT

Refined & reheated

Sunflower / palm blends used at high heat.
Oil reheated three or more times.

AVOID ENTIRELY

Trans fats

Vanaspati. Bakery shortenings. Anything
labelled “partially hydrogenated.”

MYTH TO RETIRE

“Ghee causes heart disease.” — Indian CVD is driven by diabetes, BP, smoking, ApoB, trans fats and repeated deep-frying. Not a spoon of ghee on dal.

THE MOVE

Pick two oils and rotate them. Today — throw out anything with “vanaspati” or “partially hydrogenated” on the label.

Micronutrients Indians routinely miss

Macros get attention. Micros silently decide immunity, energy and mood.

NUTRIENT	INDIAN PICTURE	FOOD-FIRST SOURCE
Vitamin D	46.5% deficient (Metropolis, 22 L tests)	fatty fish, egg yolks, sun + test
Vitamin B12	vegetarian risk	eggs, dairy, fortified, supplement
Iron	57% of women 15–49 anaemic (NFHS-5)	dal, ragi, leafy + lemon, eggs
Calcium	low across diets	dairy, ragi, til, leafy greens
Magnesium	depleted soils	nuts, seeds, whole grains, leafy
Omega-3	low in vegetarian diets	flax, walnuts, chia, small fish

Random “immunity” supplements without a blood test are expensive urine.

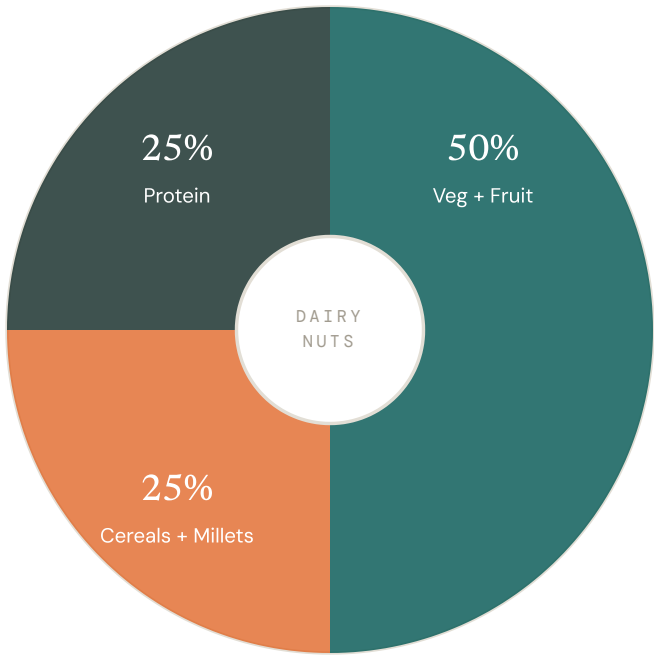
THE MOVE

Once a year, test Vitamin D, B12, ferritin and TSH. Food first. Test next. Supplement only what’s actually low.

The ICMR-NIN 2024 plate

India’s official plate template — ten food groups, seventeen rules.

VISUAL DEFAULT



WHAT’S NEW IN 2024

- Greater emphasis on millets & pulses
- Explicit restriction on ultra-processed food
- Salt < 5 g/day · Sugar < 5% of calories
- Tailored advice for women, kids, elderly, veg diets

THE TEN FOOD GROUPS

Cereals · Pulses · Meat/Fish/Eggs · Milk & dairy · Nuts & oilseeds · Fats & oils · Vegetables · Green leafy · Fruits · Sugars

THE MOVE

Download the ICMR–NIN 2024 plate PDF. Pin it on the fridge. Use it as the visual default for every meal — not as a calorie chart.

Optimizing the Indian thali

Five small upgrades convert a typical Indian meal into a near-optimal one.

01

Add sprouts

or legumes · once a day

02

Swap rice

to millets, parboiled, hand-pounded

03

Double the veg

halve the rice

04

Add dahi

or chaas to most meals

05

Finish with

saunf or jeera for digestion

REGIONAL UPGRADES

North

Roti + dal + sabzi + dahi — add salad & sprouts.

South

Rice + sambar + thoran + curd — rotate ragi in.

East

Rice + dal + small fish + shukto — keep the fish.

West

Bhakri + dal + shaak + chaas — add chana.

THE MOVE

For one week, force a salad or sprout bowl at the start of lunch and dinner. The rice portion shrinks on its own.

The processed food trap

UPF bypasses your body’s “I’m full” signal. The classic NIH trial found people ate 500 more calories per day – without trying.

HIDDEN UPF · LOOKS HEALTHY

- Atta biscuits & rusks
- Flavoured yogurts
- Instant noodles, masala oats
- Sugary breakfast cereals
- “Baked” chips, protein bars
- Cold drinks, packaged juice

NOVA / ICMR-NIN 2024 framework

+500

extra kcal/day on UPF vs whole food — matched for calories, fat, sugar & fibre.

Hall et al., Cell Metabolism 2019

PRACTICAL FILTER

If a packet has > 5 ingredients you wouldn’t find in a home kitchen – treat it like dessert, not staple.

THE MOVE

Cap UPF at < 20% of your weekly calories. Aim for whole-food meals five days a week — packaged on the other two.

Reading a nutrition label in 20 seconds

Most labels are designed to mislead. Four habits make the back of any packet readable.

01

Read /100 g

Not “per serving” — companies pick tiny servings to make numbers look small.

02

Top 3 ingredients

Listed by quantity. If sugar is in the top 3, that product is mostly sugar.

03

Hidden sugars

Maltodextrin, dextrose, glucose syrup, invert sugar, cane juice solids — all sugar.

04

Real serving size

“100 ml” on a 500 ml bottle hides the real numbers. Compute the real intake.

MARKETING WORDS THAT DON’T MEAN HEALTHY

“Fortified” · “Natural” · “No added sugar” (still has juice concentrate) · “Multigrain / atta” (often < 10% whole grain) · “Trans fat 0 g” (anything < 0.5 g/serve can be zero by law)

THE MOVE

On your next grocery run, read 5 labels. Put back anything where sugar is in the top three ingredients.

Sugar — the real story

WHO ceiling: free sugars < 5% of calories — about 25 g on a 2000 kcal day. Most Indians blow past it by chai-time.

CHAI MATH FOR A REGULAR DAY

4 cups chai × 2 tsp	32 g
3 biscuits	12 g
1 cup Bournvita milk	14 g
2 tbsp ketchup	8 g
Daily total	66 g

~ 2.6× the WHO 5% ceiling — before dinner.

FRUIT ≠ FRUIT JUICE

1 apple

~10 g sugar · with fibre & water. Slow absorption.

250 ml apple juice

~25 g sugar · zero fibre. Spikes like a soft drink.

CRAVINGS

Cravings genuinely fade by day 14 of cutting one sugar source cold turkey.

THE MOVE

Pick one sugar source — chai, biscuits or juice — and remove it for 30 days. Just one.

Hydration — ignore the 8-glass rule

It has no scientific basis. Your body has a far simpler biomarker.

URINE COLOR · YOUR FREE DASHBOARD



Pale straw
Hydrated. Carry on.



Yellow
Drink a glass.



Amber or darker
Drink more. If persistent, see a doctor.

ELECTROLYTES ≠ HYDRATION

Outdoor workers, summer commuters and 35°C+ exercise need sodium — not just water. ORS mixed exactly per packet · salty food + fluids · proper electrolyte drink.

REASSURANCE

Caffeine’s mild diuretic effect doesn’t dehydrate you. Food provides 20–30% of daily fluid. Over-drinking is rare but possible in extreme exercise.

THE MOVE

Carry a 1-litre bottle as your daily scaffold. Check urine colour as your gauge. Add electrolytes in summer or after hard workouts.

Cooking and meal prep

One hour on Sunday — five weekday dinners decided before Monday morning.

THE SUNDAY HOUR

- 1. Pressure-cook one large batch of dal.
- 2. Pre-chop & store 3 different sabzis.
- 3. Ferment idli/dosa batter or set overnight oats.
- 4. Roast a tray of peanuts, chana & seeds.

~ 60 minutes. Five evenings handled.

PRESERVE NUTRIENTS

Steam · pressure-cook · tawa-roast · quick stir-fry with lid. Deep-fry occasionally — never with reheated oil.

AVOID

Oil reheated 3+ times — rising trans fats and polar compounds. Repeated microwaving of the same dish.

QUICK WINS

Overnight oats with banana + jaggery. Eggs + leftover sabzi = bhurji. Curd-based marinades for tenderising & probiotics.

THE MOVE

Block 60 minutes every Sunday evening as “prep hour” on the family calendar. Less cooking skill, more scheduling discipline.

PHASE 03 / 08

BODY & MIND 05

Gut & Metabolic Health

TOPICS 19 – 25 ——— *The engine room. Microbiome, fibre, glucose, insulin – and the diabetes epidemic.*

Your second brain — the gut microbiome

Trillions of microbes — roughly 1:1 with human cells — running 70% of your immune system.

INDIA’S STRUCTURAL ADVANTAGE

A fermented-food culture, already.

Idli, dosa, dhokla, dahi, kanji, lassi, naturally fermented pickles. Some deliver live microbes (fresh curd, kanji); others improve nutrient bio-availability through fermentation.

INDIA’S STRUCTURAL DISADVANTAGE

Antibiotic abuse, low-fibre urban diets.

OTC antibiotics, processed snacks, declining diet diversity, rising C-sections and formula feeding — each erodes microbial diversity.

FOUR FEEDS FOR THE GARDEN

Daily fermented

Dahi at minimum

30+ plants/week

Diversity feeds diversity

No casual antibiotics

Only when prescribed

Breastfeed

Where possible

THE MOVE

Count plant foods in your week — vegetables, fruits, dals, nuts, seeds, whole grains, herbs. Aim for 30+ different plants per week.

Fibre — the 30-gram floor

Cheap. Available everywhere. Most urban Indians get half of what they need.

A SAMPLE DAY THAT HITS 33 G

Breakfast	
Oats + chia + fruit	~10 g
Lunch	
Dal + salad + sabzi	~10 g
Snack	
Roasted chana + apple	~5 g
Dinner	
Millet roti + sabzi	~8 g
Total	~33 g

SOLUBLE

Oats, dal, fruits, isabgol. Feeds gut bacteria, slows glucose absorption, lowers LDL.

INSOLUBLE

Whole grains, vegetables, seeds. Adds stool bulk, speeds transit time.

DON'T DO THIS

Don't jump from 15 g to 40 g overnight. Add ~ 4–5 g per week. Add water alongside.

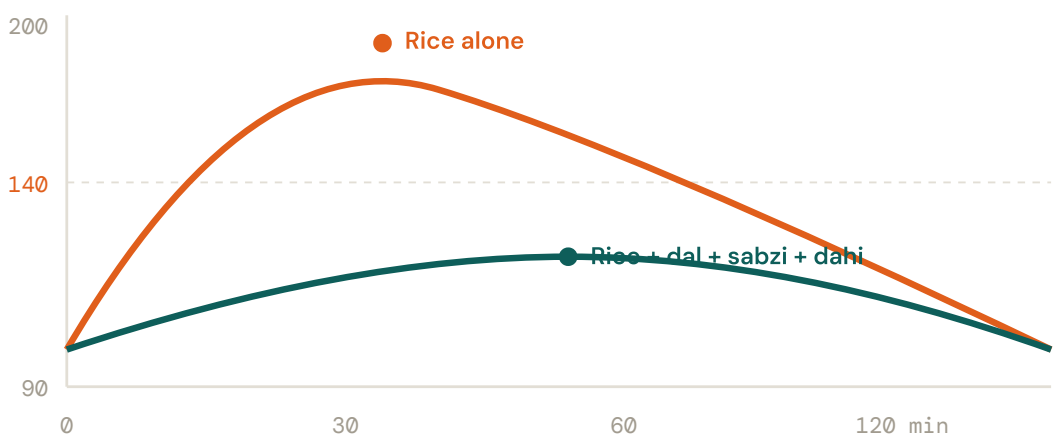
THE MOVE

30 g is the daily floor. Keep a sachet of isabgol as the backstop on low-fibre days.

Blood sugar 101

Same calories. Different curves. Sequence & sides decide the spike.

POST-MEAL GLUCOSE CURVES (illustrative)



Should return below 140 mg/dL by the 2-hour mark.

EAT IN THIS ORDER

Veg → Protein → Carbs

Meal sequence can flatten the spike meaningfully — varies by person and meal.

CGMS · A NUANCED VIEW

Transformative for many on insulin or tight monitoring. For pre-diabetic curiosity, a 10–14 day trial educates — but diagnosis still relies on standard blood tests. Optimize the diet, not the graph.

THE MOVE

Two free wins: veg/protein before carbs at every meal, and a 10-minute walk after eating.

Insulin resistance — the engine

The disease before the disease. Precedes Type 2 diabetes by 5–10 years.

SIGNS IN BLOOD

- Fasting insulin
- HOMA-IR
- Triglyceride / HDL ratio
- Rising HbA1c trend

SIGNS ON THE BODY

- Waist-to-height > 0.5
- Acanthosis nigricans (dark neck folds)
- Skin tags
- PCOS · fatty liver

DRIVERS · ALL MODIFIABLE

- Visceral fat
- Poor sleep
- Chronic stress
- Refined-carb-heavy diet
- Low muscle mass

You can be thin and still insulin-resistant — the thin-fat Indian phenotype again.

THE MOVE

Test the core panel — fasting glucose, HbA1c, triglycerides, HDL, waist, BP. If markers drift, walk after meals, lift 2×/week, sleep 7+, cut UPF. Retest in 3 months.

Metabolic health in five numbers

Three out of five abnormal $\Rightarrow$ metabolic syndrome. CVD risk doubles, diabetes risk multiplies.

01

Waist

M < 90 cm
W < 80 cm

02

Fasting glucose

< 100
mg/dL

03

Triglycerides

< 150
mg/dL

04

HDL

$$\begin{aligned} M &> 40 \\ W &> 50 \end{aligned}$$

05

Blood pressure

< 130/85
mmHg

WHY THESE FIVE

Each catches a different failing system — fat storage, glucose handling, lipid metabolism, blood vessel health. All available at any Indian path lab for ~ ₹500.

THE MOVE

Measure all five at your next visit. Tape them to the fridge. Re-measure every six months — the trend matters more than any single reading.

India’s diabetes epidemic

A quarter of a billion people with diabetes or pre-diabetes. And a remarkable evidence base for prevention.

101 M

living with diabetes today.

ICMR-INDIAB / Lancet 2023

136 M

pre-diabetic. The largest in the world.

ICMR-INDIAB / Lancet 2023

58%

drop in progression with lifestyle alone.

DPP trial · NEJM

PROJECTION

Age-standardised diabetes prevalence projected to rise from ~6,150 per 100,000 (2022) to ~8,585 by 2031 if current trends hold.

Frontiers in Endocrinology · 2025

THE HOPE

For newly diagnosed (< 5 years), structured weight loss has produced meaningful remission rates — Mohan & Pradeepa, Newcastle, Virta protocols.

THE MOVE

Pre-diabetic? The lifestyle protocol is non-negotiable. Newly diagnosed? Ask your endocrinologist about remission — not just management.

Intermittent fasting

Similar to plain caloric restriction. Useful, not magic.

WINDOWS · START EASY

12:12

Eat 8 am–8 pm

EASY

14:10

Eat 9 am–7 pm

STANDARD

16:8

Eat 12 pm–8 pm

AGGRESSIVE

BMJ meta-analysis · June 2025 (99 RCTs)

DO NOT FAST IF

- Pregnant or breastfeeding
- Type 1 diabetes (without endocrinologist)
- History of eating disorder
- PCOS or hormonal cycle issues — be cautious
- Sleep, mood, training or cycle worsens — step back

THE MOVE

Start with 12:12 for two weeks. If sleep, energy and hunger improve, extend. If no benefit at six weeks — it’s not your tool.

Movement & Fitness

TOPICS 26 – 32 ——— *The most powerful longevity drug we have — and how to actually dose it.*

Exercise as medicine

If a pill could do this, it would be the most prescribed drug in the world. Peter Attia calls it the single most powerful longevity drug we have.

THE WHO PRESCRIPTION

Moderate cardio	150–300 min/wk
Vigorous cardio	75–150 min/wk
Strength training	2 sessions/wk
Daily breaks	every 30 min

CONDITIONS EXERCISE LOWERS RISK FOR

40+

T2D, CVD, dementia, depression, anxiety, multiple cancers, osteoporosis, sarcopenia, sleep apnea, hypertension.

“Housework counts as exercise.” The data does not agree.

THE MOVE

Block 3 cardio + 2 strength sessions per week as recurring calendar entries. Make it boring before you make it fancy. Consistency beats intensity.

Cardio – Zone 2 and Zone 5

Most beginners exercise at “medium-hard” – neither easy enough for volume nor hard enough for peak. Two zones do the actual work.

ZONE 2 · CONVERSATIONAL

60–70%

of max heart rate

Builds mitochondria, improves fat oxidation. Brisk walk, cycle, swim.

TARGET

~ 3 hrs / week

ZONE 5 · ALL-OUT

90–100%

of max heart rate

Raises VO₂ max — one of the strongest predictors of all-cause mortality.

PROTOCOL

4 × 4 min hard / 3 min easy

THE MOVE

Three Zone 2 sessions a week + one Zone 5 interval session. That is the entire cardio program for most adults.

Strength training

Possibly India’s most under-prescribed medicine — especially for women.

10–20%

drop in all-cause mortality from just 30–60 min of resistance training per week.

British J of Sports Medicine · meta-analysis

~30%

drop in CV mortality in women specifically.

JACC · 2024

MYTH

“Lifting makes women bulky.” — Building a very muscular physique takes years of intentional training and genetics. For most women, lifting makes them stronger, leaner, and independent through their 60s, 70s, 80s.

BEYOND MORTALITY

- Stronger bones — critical given Indian Vit-D & calcium gaps
- Prevents sarcopenia — the silent age tax
- Direct counter to insulin resistance
- Better balance, lower fall risk in elders

THE MOVE

Two sessions per week. 30–45 minutes each. Bodyweight is enough to start — squats, push-ups, rows, hinges, carries.

The five movement patterns

One exercise per pattern – full-body workout in 30 minutes, no gym.

01

Squat

Sit down, stand up

Bodyweight squat · goblet squat · chair sit-stand

02

Hinge

Bend at hips, flat back

Romanian deadlift · kettlebell swing · good morning

03

Push

Press load away

Push-up · wall press · overhead press

04

Pull

Draw load to body

Inverted row · door row · band row

05

Carry

Walk loaded

Suitcase carry · farmer’s carry · overhead carry

PROTOCOL 1 set each, 8–12 reps, 2–3 rounds TIME ~ 30 minutes FREQUENCY 2 sessions/week

THE MOVE

Cover all five patterns twice a week and you’ve completed the WHO strength target. No gym membership required.

Walking – the most underrated

Free. Joint-friendly. Available outside almost every Indian house.

STEP COUNT · MORTALITY

~4,000	benefit starts
7–10 k	the sweet spot
12 k+	diminishing returns

And speed matters — brisk beats slow shuffle.

THE 10-MIN POST-MEAL WALK

12–22%

smaller post-meal glucose spike. The cheapest blood-sugar intervention in existence.

INDIAN CONTEXT

Society loops · indoor mall walks in summer · treadmill at home · walking meetings at work.

THE MOVE

Two non-negotiable walks — 10 minutes after lunch and 10 minutes after dinner. Then add a longer brisk walk 3 times a week.

Mobility, posture and ergonomics

India’s IT desk culture is silently rewiring people’s bodies.

THE 5-MINUTE NIGHTLY RESET

- 1. Hip flexor lunge stretch — 60 s each side
- 2. Thoracic extension over a chair back — 60 s
- 3. Doorway chest stretch — 60 s each side
- 4. 10 glute bridges

Persistent pain or numbness? See a physiotherapist — don’t stretch through it.

THE DESK RULE

20 · 8 · 2

Sit 20 min · stand 8 min · move 2 min. Stack with chai breaks.

WFH SETUP AUDIT

Laptop top of screen at eye level · external keyboard · chair with lumbar support · feet flat on floor.

THE MOVE

Do the 5-minute reset before brushing your teeth at night. Every night. Two weeks in, your back will thank you.

Recovery is not laziness

You don't get stronger during the workout. You get stronger between workouts.

PILLAR 01 · SLEEP

7–9 hrs

Where the actual physical rebuild happens.

PILLAR 02 · PROTEIN

1.2–1.6 g/kg

Raw material for repair. Spread across meals.

PILLAR 03 · DELOAD

every 4–6 wk

Drop volume ~30%. Growth happens in the gaps.

UNDER-RECOVERED, NOT UNDER-TRAINED — LOOK FOR

RHR rising over weeks · sleep worse on training days · mood dropping · persistent niggles · motivation disappearing.

THE MOVE

Schedule two full rest days a week. Drop training volume by ~30% every fifth week. Trust the cycle.

PHASE 05 / 08

BODY & MIND OS

Sleep, Stress & Mind

TOPICS 33 – 38 ——— *Active maintenance — for the body, the nervous system, and the family.*

Sleep science 101

Sleep is not rest. Sleep is active maintenance — 4–6 cycles of ~90 minutes each, every night.

ONE SLEEP CYCLE

N1 + N2 · light Transition. Lays the groundwork.	~50%
N3 · deep (slow wave) Physical repair. Glymphatic clean-up.	~25%
REM Memory consolidation, emotional processing.	~25%

7–9 h

for adults · consistently. Weekend sleep-ins do not refund the weekday debt.

EVEN ONE BAD NIGHT

Disturbs insulin sensitivity, hunger and blood sugar. Chronic short sleep compounds into weight gain, hypertension, cognitive decline and depression.

THE MOVE

Block 8 hours of sleep opportunity nightly — lights off, phone away. Same wake time every day, weekends included.

Sleep hygiene – Indian edition

Generic advice assumes a quiet, dark room and early dinner. India is the exception, not the norm.

INDIAN SLEEP KILLERS

- 10 pm dinners → reflux, delayed melatonin
- Street light through thin curtains
- Joint family disturbances, infants nearby
- Fan-vs-AC trade-offs in summer heat
- Late-night chai + phone scrolling

LOCALIZED FIXES

- Dinner 2–3 hrs before bed, lighter & less spicy
- Blackout curtains or sleep mask
- 22–24 °C room temp · fan with light cover
- Phone charged in another room
- No overhead lights after 9 pm

THE CHAI MATH NOBODY DOES

Caffeine half-life is *5–7 hours*. A 5 pm chai leaves ~50% of its caffeine in your system at 10 pm.

THE MOVE

Change one thing at a time. Move dinner earlier → phone out of bedroom → no caffeine after 2 pm. Two weeks per change.

The stress response

Cortisol evolved for tigers – not EMIs, deadlines and family WhatsApp groups.

CHRONIC STRESS · THE BODY KEEPS SCORE

- Visceral fat gain
- Insulin resistance
- Higher blood pressure
- Weaker immune function
- Poor sleep & worse food choices

INVISIBLE BECAUSE HUSTLE IS NORMALIZED

- Tense shoulders, jaw clenching
- Belly fat that won’t shift
- “Tired but wired” at night
- Digestive issues, frequent illness
- Short fuse with family

MINIMUM VIABLE STRESS REDUCERS

Daily movement · 5 minutes of breathwork · one full day off the phone per week.

THE MOVE

If any of those three feels impossible to fit in — that itself is the loudest signal about your current stress load.

Breathing techniques that actually work

The only autonomic function you can also drive manually.

TECHNIQUE 01

Physiological sigh

2 inhales (nose) → 1 long exhale (mouth)

Best for: in-the-moment reset before a meeting, during an argument, when you can't sleep.

Stanford · Huberman et al., 2023

TECHNIQUE 02

Box breathing

Inhale 4 · hold 4 · exhale 4 · hold 4

Best for: pre-task focus. Navy SEAL reset. 4–8 rounds.

TECHNIQUE 03

Nadi shodhana

Alternate nostril breathing

Best for: morning or evening practice. Surprisingly well-supported by HRV research.

Five minutes a day, compounded over weeks, can measurably move stress, mood, HRV, sometimes BP and sleep onset.

THE MOVE

Stack with existing rituals. Every chai break — 4 rounds of box breathing. Every bedtime — 5 minutes of nadi shodhana.

Mental health basics for Indian families

Not therapy training. Just enough literacy to recognize, talk, and escalate before a crisis.

SADNESS vs DEPRESSION

Sadness

Event-triggered · time-limited · responds to change.

Depression

Persists 2+ weeks · hopelessness · functioning drops. See a professional.

WORRY vs ANXIETY

Worry

Specific · about a problem · usually time-bound.

Anxiety

Diffuse · body-felt · intrusive · hard to switch off.

SAVE THESE NUMBERS

Tele-MANAS

14416

also 1800-891-4416

Vandrevala Foundation

+91 9999 666 555

24×7

iCall

9152987821

Mon–Sat 10 am–8 pm

THE MOVE

Save the three helplines in every family phone. Say “I’m seeing a therapist” the way you’d say “I’m seeing a dentist.”

Habit formation

Every recommendation in this curriculum dies on impact without a habit framework. Willpower is finite. Systems are infinite.

THE LOOP · DUHIGG



Design the cue and the reward — the routine takes care of itself.

BJ FOGG'S ADDITION

Start absurdly small. Two push-ups. One paragraph. Build the neural pathway first — dose second.

HABIT-STACKING FOR INDIAN HOMES

Brush teeth	→	10 squats
Pour chai	→	10 deep breaths
Sit for dinner	→	1 raw veg first
After lunch	→	10-min walk

THE MOVE

Pick ONE habit. Make it tiny. Anchor it to something you already do daily. Track 30 days. Only then add the next.

Indian-Specific Challenges

Vitamin D, PCOS, premature CVD, women, kids, elders – the gaps that hit Indian bodies harder.

The Vitamin D deficiency epidemic

A sun-drenched country with one of the world’s worst Vitamin D pictures.

46.5%

deficient · < 20 ng/mL

Metropolis Labs · 22 L tests, 2019–Jan 2025

26%

insufficient (20–30 ng/mL)

Same dataset

~96%

of pregnant Indian women deficient in some studies.

Indian J of Endocrinology & Metabolism

THREE LEVERS · INDIVIDUALIZE

Sun

Safe exposure — dose varies by skin tone, season, pollution, age.

Food

Fatty fish · egg yolks · fortified milk.

Supplement

Correct first · maintain 800–2000 IU/day, retested.

THE MOVE

Test 25-hydroxy Vitamin D annually. If deficient, correct with a high-dose regimen first, then maintain. Possibly the highest-ROI health intervention in India today.

The PCOS problem

Common, under-managed, and metabolic – not just gynaecological.

PREVALENCE

NIH criteria	7.2%
Rotterdam criteria	19.6%
Delhi NCR 2025	17.4%

JAMA Network Open · multicentre Indian

METABOLIC LOAD AMONG DIAGNOSED

Obesity	43%
Dyslipidemia	92%
NAFLD	33%

Insulin resistance is a major driver in many cases — especially with weight gain, fatty liver, high triglycerides, or pre-diabetes.

FIRST-LINE PROTOCOL

5–10% weight loss where needed · strength training 2×/week · low-glycemic eating · sleep · stress management. Metformin / OCP / inositol decisions belong with a specialist — alongside lifestyle, not instead of it.

THE MOVE

PCOS is metabolic. Treat the body as a whole — not just the cycle — with a gynaecologist or endocrinologist coordinating.

Heart disease in young Indians

CVD hits Indians ~ 10 years earlier than Western populations. Premature heart attacks are not rare anymore.

INDIAN HEART ASSOCIATION

50%

of Indian male heart attacks happen before age 50.

25%

happen before age 40.

MEASURE THESE BEFORE 30

- ApoB — better than total cholesterol
- Lipoprotein(a) — one-time, genetic, elevated in many South Asians
- Blood pressure (averaged)
- HbA1c & fasting glucose
- Waist circumference
- Tobacco status (smoke + chewing)

Heart disease is a decades-long process. The prevention has to be decades-long too.

THE MOVE

Over 30? Full lipid panel including ApoB — non-negotiable. One-time Lp(a). If Lp(a) is high, aggressive risk-factor control for life.

The Indian diabetes playbook

HbA1c alone is not the full picture. And the story isn't about sugar – it's about insulin resistance.

THE FULL PANEL

- HbA1c + fasting glucose
- Waist circumference
- Blood pressure (averaged)
- Triglycerides & HDL
- Full lipid panel
- If risk is high: post-meal glucose, OGTT, fasting insulin (with clinician)

PREVENTION PROTOCOL · DPP-BACKED

58% ↓

progression to T2D with lifestyle alone.

- 5–7% weight loss
- 150 min/week of moderate exercise
- Refined carbs → millets, dal, veg
- 25–30 g/day fibre · 7–9 hrs sleep

FOR NEW DIAGNOSES · WITHIN 5 YEARS

Remission is real. Mohan & Pradeepa, Newcastle, Virta protocols — with medical supervision.

THE MOVE

Pre-diabetic? The lifestyle protocol is your contract with future-you. Newly diagnosed? Find a clinician open to remission — not just “manage forever.”

Women’s health essentials

Five themes that get under-prioritised – even inside Indian families.

01

Iron & anaemia

57% of women 15–49
anaemic. Test Hb, ferritin, TIBC.
Treat with food + targeted
supplementation.

02

Cycle as 5th vital sign

Length, regularity, flow, pain
are clinical signals. Sudden
changes deserve investigation.

03

Fertility basics

Decline accelerates after 35.
Knowing the ovulation window
removes anxiety and
unnecessary delay.

04

Peri- & menopause

Lasts ~10 years. Bone density
screening becomes important.
HRT decisions belong with a
specialist.

05

Pregnancy nutrition

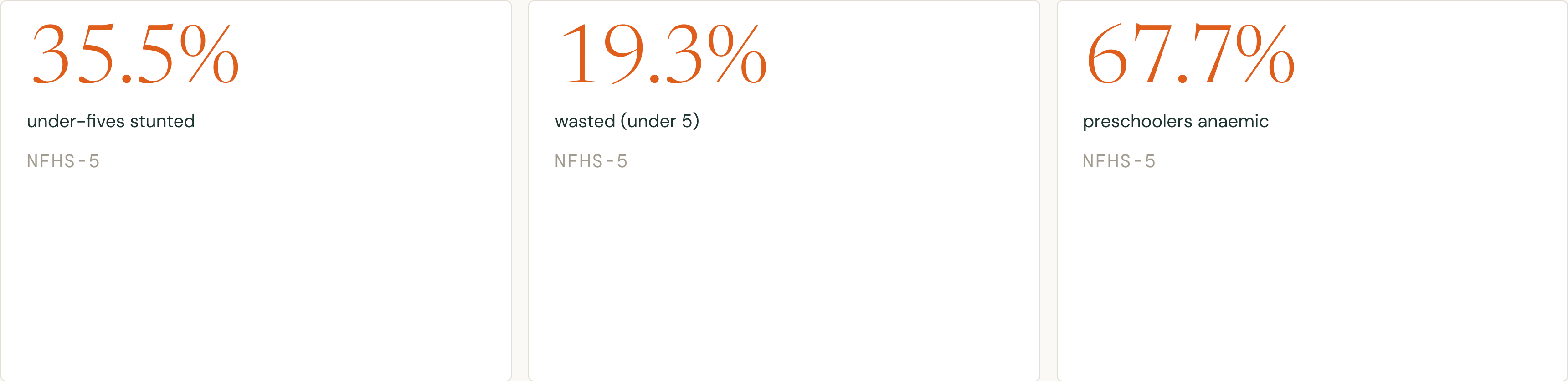
Folate 3 months pre-
conception. Iron, B12, Vit D,
calcium, iodine. First 1,000
days set the trajectory.

THE MOVE

Period-tracking apps are the cheapest health diary a woman can keep. Pair with annual ferritin and TSH. Discuss bone density after menopause.

Children’s nutrition and growth

India simultaneously battles stunting and rising childhood obesity. The first 1,000 days set the trajectory.



FIRST 1,000 DAYS
Exclusive breastfeeding to 6 months · then diverse complementary foods — *not packets*. Sets metabolic, cognitive and immune trajectory.

MYTH
“Bournvita is protein.” It’s candy dissolved in milk. Real protein is dal, eggs, paneer, fish, chicken.

THE MOVE Track growth curves, protect sleep, and give kids at least two hours of outdoor time daily — the cheapest myopia, mood, and bone-density intervention there is.

Elder care basics

India is aging quietly but fast. Five fronts to monitor for anyone past 60.

01

Sarcopenia

Muscle loss begins ~ 30, accelerates after 60. Strength training + 1.2–1.6 g/kg protein. Independence vs dependence.

02

Bone density

Crashes post-menopause. DEXA after 60. One hip fracture often ends mobility permanently.

03

Polypharmacy

5+ meds = compounding side effects. Quarterly review with one doctor — every “lifelong” prescription justified.

04

Vaccines

Annual flu · pneumococcal per doctor · shingles 50+ · Covid boosters · Tdap every 10 yrs.

05

Loneliness

Health risk on par with smoking. Check in on mood, not just medicines.

THE MOVE

Add a quarterly “parent check-in” to the family calendar — medication review, fall-prevention scan of the house, and a real conversation about mood.

Screening & Toolkit

TOPICS 46 – 52 ——— *What to test, when, and how to read the report — the most practical phase.*

Preventive screening calendar by age

Most lives lost to NCDs in India were lost to a missing entry on a calendar.

AGE	PRIMARY TESTS	SECONDARY	ADD-ONS / NOTES
20s BASELINE	Discuss baseline with a doctor based on diet, family history & risk.	BP · dental · eye (if needed) · mental health check-ins	Targeted: CBC, glucose, lipids, TSH, Vit D, B12 if at risk
30s ADD	Routine BP. HbA1c if risk factors. ApoB & one-time Lp(a) if family history.	LFT/KFT per risk · clinical breast exam (women)	Cervical screening from ~30 per local guidance (VIA / HPV / Pap)
40s ADD	ECG / 2D echo if CV risk. From 40, mammogram every 2 yrs (USPSTF 2024).	From 45 — colorectal screening (FIT / colonoscopy)	Annual eye exam · DXA for post-menopausal women with risk
50+ FULL ANNUALS	BP · diabetes · lipid review every year. Comprehensive eye check.	Vaccines · flu, shingles, pneumococcal, Tdap	Discuss DXA ~5 yrs after menopause or earlier if risk

THE MOVE

Match this calendar to every family member. Book the next applicable screening this week — not “next month.”

Reading your blood report

Pattern beats a single number. One borderline reading isn't disease — three correlated ones are a system drifting.

WHAT TO LOOK AT, BY PANEL

CBC	Hemoglobin · MCV · MCH
Lipid	LDL · HDL · Trig · ApoB
Sugar	HbA1c · fasting glucose
Liver	ALT · AST
Kidney	Creatinine · eGFR
Thyroid	TSH

WHEN TO WORRY

Sudden large changes or 3+ correlated flags — e.g. high fasting glucose + high triglycerides + rising waist.

WHEN TO RETEST

A single borderline number · retest in 3 months. Don't treat — verify the trend first.

WHEN TO IGNORE

An isolated trivial flag in an otherwise clean panel. Investigate. Don't catastrophize.

THE MOVE

Keep a one-page spreadsheet of your key markers, by date. Bring it to every doctor visit — the doctor should read trends, not ranges.

Cancer screenings that save lives in India

Cervical, breast, colorectal and oral — four early-detection wins. India is dramatically under-screened.

CERVICAL

HPV / VIA / Pap from age 30

Plus HPV vaccination for girls and boys 9–14. Self-sampling where HPV testing exists.

< 2%
of eligible Indian women have been screened.

BREAST

Clinical exam 1–3 yrs (ICMR)

Mammogram every 2 yrs from age 40 per 2024 USPSTF.

Self-awareness — report any new lump. Not paranoia, awareness.

COLORECTAL

FIT or colonoscopy from 45

Earlier if family history of colorectal cancer or polyps. Discuss interval with GI.

ORAL

Annual visual check

By a dentist, ENT, or trained health worker. India carries one of the world’s highest oral cancer burdens — tobacco is the main driver.

THE MOVE

Book one screening for one eligible woman in your family this month. The bar is that low. And that consequential.

Wearables and health tracking

Some of it is reliable. A lot of it is directional at best.

TRUST

- Step count — robust
- Heart rate — resting and active
- Active minutes
- ECG (atrial fibrillation detection on some watches)

DIRECTIONAL ONLY

- Sleep stages — magnitude right, specifics often off
- SpO₂ — trend > absolute
- HRV — daily readiness gauge if tracked over weeks
- Calorie burn — usually wildly off

CGMS · A NUANCED VIEW

For many with diabetes (especially on insulin), CGMs are transformative. For pre-diabetic curiosity, a 10–14 day trial educates — but diagnosis still relies on standard blood tests. Optimize the diet, not the graph.

THE MOVE

Pick two metrics — daily steps + resting heart rate. Ignore the rest. Watch trends across months, not days.

Eye health and screen time

Digital eye strain is real but not blinding. The bigger Indian story is child myopia and unscreened diabetic retinas.

THE 20 · 20 · 20 RULE

Every 20 min, look at something 20 ft away for 20 s.

Reduces accommodative strain. Blink consciously — screen use halves blink rate.

EXAMS THAT MATTER

- Annual eye exam from age 40
- Diabetics — yearly retinal exam
- Glaucoma check from 40 (earlier with family history)
- Kids — refraction by age 6

CHILD MYOPIA SURGE

Antidote is shockingly simple — 2+ hours of outdoor time daily. Cheap. Protective. Underused.

THE MOVE

Diabetics — book a dilated retinal exam this year. It is preventable but invisible until it’s advanced. Don’t skip.

Dental health equals heart health

Gum disease is part of the wider inflammatory risk picture — not just cosmetic.

DAILY BASICS

- Brush twice daily · 2 min · soft brush
- Floss daily — matters more than mouthwash
- Clean the tongue
- Skip sugary mouth rinses & whitening pastes

THE SCHEDULE

- Twice-yearly dental scaling + check-up
- First child dental visit by age 1
- Orthodontic check at 7
- Tobacco users — yearly professional oral exam

Periodontitis is independently associated with heart disease risk and diabetes control. Twice-yearly scaling is cheaper than one root canal — and possibly one of the best inflammation interventions you can buy.

THE MOVE

Add dental appointments to the family health calendar — quarterly review window picks them up automatically.

Skin health fundamentals

Three steps that work. Everything else is optional — or marketing.

01

Cleanse

Twice daily · gentle cleanser · lukewarm water. No bar soap on the face.

02

Moisturize

Per skin type. Hot, humid climates · light gel. Dry winter skin · thicker cream.

03

SPF 30+

Daily — even on brown skin. PA+++ UVA coverage. Reapply every 2–3 hrs outdoors.

AVOID

Steroid–mix creams without a derm · over–the–counter “skin lightening” products · dermat–influencer routines with 8+ products. *Skin is medical. The algorithm is not.*

THE MOVE

Build a three–step routine — cleanse, moisturize, SPF. For anything beyond that, see a dermatologist — not Instagram.

Family Health OS

TOPICS 53 – 60

Captain, vault, calendar, insurance — the operating system that keeps the previous 52 topics from being skipped.

Family health management

Every Indian family has an unofficial health captain. Make the role explicit, shared, and tooled.

ROLES

Captain	Owens calendar & vault
Co-captain	Backup — prevents single-person failure
Members	Own their own appointments & meds
Children	Progressively own health by age 18

RITUALS

Weekly	30-min Sunday review
Monthly	Weigh-in, waist, BP per adult
Quarterly	Dental, eye, or derm check-ins
Annually	Full blood panels & screenings

THE MOVE

Hold a 30-minute “family health constitution” meeting this Sunday. Pick a captain and a co-captain. Create the shared calendar. That meeting — not any app — is where the OS begins.

The family health vault

A 30-minute Sunday project that saves lives during emergencies.

PER PERSON

- Medical history · chronic conditions, surgeries
- All allergies — drug, food, environmental
- Blood group
- Current medications & doses
- Last 3 years of major reports

FAMILY-LEVEL

- Insurance policy numbers + claim contacts
- 3 emergency contacts — one outside the city
- Family doctor & specialists’ contacts
- Organ donor wishes / advance directives
- ABHA / digital backup

Digital beats paper. ABHA, shared encrypted cloud folder, or a family health app — backed up and accessible from any phone.

THE MOVE

Block one Sunday afternoon. Pull every cupboard report into one indexed folder. Share access with the co-captain. Update every six months.

Mental health for the family

The aim isn't to make every parent a therapist. It's to make conversations safe enough that the right help gets sought in time.

WARNING SIGNS

Children

Withdrawal · regressed behaviour · sleep changes

WARNING SIGNS

Teens

Falling grades · isolation · signs of self-harm

WARNING SIGNS

Adults

Persistent low mood · increased alcohol · loss of interest

WARNING SIGNS

Elders

Withdrawal · “burden” talk · sleep changes

SUICIDE PREVENTION · THE LITERACY

Asking about it does not “give someone the idea.” Listen. Don’t lecture. Take any mention seriously. Stay with the person. Call the helplines — [Tele-MANAS 14416](#) · [Vandrevala +91 9999 666 555](#) · [iCall 9152987821](#).

THE MOVE

Pin updated helplines on the fridge. Have one explicit family conversation about mental health this month — 20 awkward minutes, potentially the highest-ROI 20 minutes of the year.

Health insurance decoded

Indians are dramatically under-insured. Five concepts decode ~ 80% of the choice.

FIVE CONCEPTS

- Mediclaim vs critical illness — different things
- Family floater vs individual — floater is usually cheaper
- Room rent caps — silently shrink claims
- Pre-existing waiting period — up to 3 yrs
- OPD covers — newer, sometimes worth it

PUBLIC & TAX

- PM-JAY — ₹5 L/family/yr (SECC eligible). Check the portal.
- ESIC — for employed workers below the wage ceiling
- Section 80D (old tax regime)
 - ₹25K self/spouse/kids · ₹50K if seniors covered
 - Parents have a separate limit
 - Up to ₹5K preventive check-ups

THE MOVE

Audit your policy. Sum insured at least 10 L per adult · no room-rent cap · PED waiting period $\leq$ 3 yrs · daycare procedures included · add a critical illness rider in your 30s or 40s.

Building the annual health plan

One calendar. One budget. Twelve months. Repeat every year.

THE 12-MONTH CALENDAR

JAN Baseline blood panels – all adults	FEB Review & book follow-ups	MAR Dental scaling – whole family	APR Vaccinations · flu top-up if missed	MAY –	JUN Eye exam + skin check
JUL Mid-year health review	AUG –	SEP Age-specific screenings · Topic 46	OCT Festival prep · medications, refills	NOV Pollution / asthma readiness	DEC Recovery week, plan next year

BUDGET / ADULT / YEAR ₹1,500–5,000

CHILDREN ₹500–2,000

80D PREVENTIVE up to ₹5,000

THE MOVE

Create the 12-month calendar today. Book the January baseline labs this week. If you time-box health, it happens. “Later” in health usually means too late.

The 80/20 of health spending

Boring spending compounds. Glamorous spending evaporates.

HIGH-ROI · SPEND MORE

- Real food — fruits, dal, eggs, fish, veg
- Decent walking shoes, basic gym setup
- Good sleep environment — mattress, curtains, AC
- Annual blood work & dental visits
- Real therapy with a qualified professional

LOW-ROI · CUT

- ~~· Multivitamins without a deficiency~~
- ~~· “Detox” packages & juice cleanses~~
- ~~· Miracle gadgets ozone therapy~~
- ~~· “Immunity boosters”~~
- ~~· Boutique wellness retreats~~

Routine prevention is usually far cheaper than chronic disease treatment. Prevention is the cheapest insurance there is.

THE MOVE

Audit last year’s health spending. Move 20% of the “supplements + gadgets” budget into real food, annual labs, and therapy. Re-audit in 12 months.

Common health myths Indians believe

Six widely-held beliefs that are wrong, partly wrong, or dangerously over-applied.

MYTH

“Ghee causes heart disease.”

WHAT’S TRUE

Whole-food ghee in moderation is not the sole driver. The risk stack is diabetes, BP, tobacco, inactivity, ApoB, trans fats, repeated frying.

MYTH

“Fruit is too sweet for diabetics.”

WHAT’S TRUE

Whole fruit has fibre and water — modest spike. *Juice* is the problem. 1–2 servings of whole fruit a day are fine.

MYTH

“Milk is essential forever.”

WHAT’S TRUE

Many Indian adults are lactose intolerant. Dairy is optional, not mandatory. Curd & paneer are usually better tolerated.

MYTH

“Fasting is always good.”

WHAT’S TRUE

Not for pregnant women, T1 diabetics, eating-disorder history, or peri-menopausal hormonal issues. Topic 25.

MYTH

“Ayurveda is always safe.”

WHAT’S TRUE

Herb-drug interactions are real. Heavy-metal contamination documented in unregulated products. Use registered practitioners. Tell your allopathic doctor.

MYTH

“If Instagram says it...”

WHAT’S TRUE

A credential isn’t peer-reviewed evidence. Ask: do they treat patients? Cite sources? Are they selling the supplement they recommend?

THE MOVE

Before changing a habit on advice from any influencer, ask: who funded it, what does ICMR/WHO/NIH say, and is this an N-of-1 anecdote or actual evidence?

Capstone — the seven systems

Every Indian family needs seven systems. The OS just makes them harder to skip.

01

Nutrition rituals

Default plate · Sunday prep hour ·
fermented daily · 30 g fibre floor

02

Movement defaults

3 cardio + 2 strength weekly · 2 post-meal walks · 20-8-2 at desk

03

Sleep environment

8 h opportunity · dinner 2-3 h before bed · no caffeine after 2 pm · phone out of bedroom

04

Screening calendar

Annual baselines · age-specific screenings · vaccination tracking · quarterly review

05

Records vault

One source of truth — history, allergies, meds, last 3 yrs of reports, insurance, ICE contacts

06

Mental health norms

Dinner check-ins · therapy as routine · helplines on the fridge · suicide prevention literacy

07

Emergency protocol

Designated hospital · ambulance numbers · insurance card location · ICE in every phone

THE WORK

The OS removes friction. But the human still does the squats, eats the dal, and has the dinner-table conversation.

THE MOVE

Score your family today: which of the seven systems is missing? Pick one to install in the next 90 days. Then the next.

60 topics. 8 phases. *One operating system.*

Catch things in the silent phase. Trigger care by the calendar — not by the symptom. Run the OS for the people you love.

NOT PERFECT. NOT READY.
STILL UPLOADING.

END OF DECK

A learner sharing what I found — not a doctor.

These notes are compiled from *ICMR, WHO, NIH, peer-reviewed journals, and official Indian government data*. Nothing here replaces a clinician who knows you. For diagnosis, dosing, and treatment decisions — speak to a qualified professional.

KEY SOURCES CITED

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